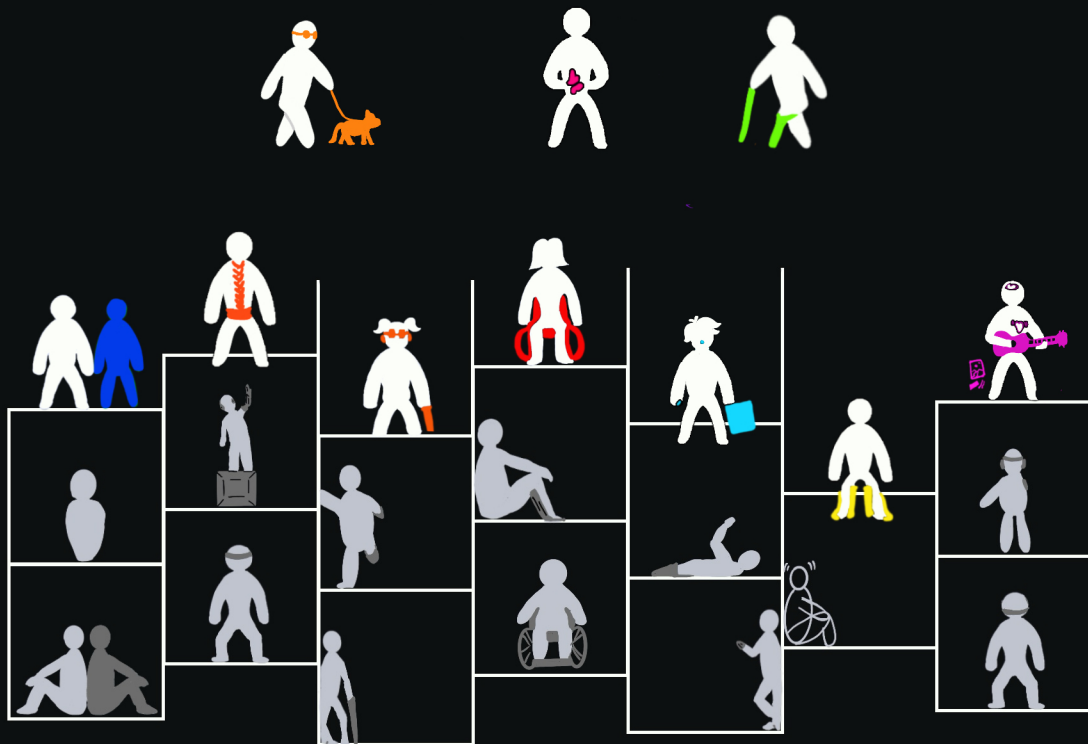


FINDING **SIZES** FOR **ALL**

A Report on the **Status** of the
Right to Accessibility in **India**



CENTRE FOR DISABILITY STUDIES
NALSAR UNIVERSITY OF LAW, HYDERABAD

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Right to Accessibility in India

Submitted to the Supreme Court of India

by the Centre for Disability Studies
NALSAR University of Law, Hyderabad

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Executive Summary

This report on the status of accessibility rights for persons with disabilities in India reveals a complex landscape of progress and persistent challenges. In the matter of *Rajive Raturi vs Union of India*, WP (Civil) 243/ 2005, the Supreme Court by its order dated November 29, 2023, designated the Centre for Disability Studies, NALSAR, Hyderabad (CDS) to inquire into the matter of Accessibility Rights of persons with disabilities in India. In a follow-up order dated January 13, 2024, the Court allowed CDS to adopt the most suitable procedure to undertake the study, including consulting subject experts, disabled people and their organisations, and sought the submission of a comprehensive report within six months.

Accessibility is not merely a standalone right, but a fundamental prerequisite for persons with disabilities to exercise all other rights. The report addresses both prongs of accessibility initiatives: the ones which rectify past exclusionary practices and as well as those which ensure inclusion in the future. The Rights of Persons with Disabilities Act 2016 (RPDA) has normatively acknowledged the importance of accessibility but implementation remains inconsistent and often inadequate. The RPDA has made it mandatory to comply with the rules on accessibility incorporated in the RPD Rules. However, the standards incorporated in the Rules are not non-negotiable compulsory standards, but only persuasive guidelines. The policy of the RPDA is not followed in the Rules. The Act intended to use compulsion to ensure accessibility, but the Rules have transformed this into self-regulation. This self-regulatory approach depends upon voluntary compliance, since people cannot be compelled to follow the guidelines. The absence of compulsion in the Rules is contrary to the intent of the Act. This lack of enforcement in the Rules, the report asserts, compromises the realisation of accessibility rights and is *ultra vires* the RPDA.

Through surveys, expert interviews, and first-person accounts, the report documents the accessibility barriers in various sectors:

Courts: A survey of High Courts found that while some accommodations exist for persons with locomotor disabilities, there is limited awareness of needs for other disabilities. District courts, especially in rural areas, face significant accessibility challenges.

Prisons: While some basic facilities are provided, there is a lack of recognition for all categories of disabilities and inadequate staffing, particularly for mental health support.

Education: A survey of government schools revealed significant barriers, including inaccessible classrooms, toilets, and learning materials. Higher education faces similar challenges, with accessibility influencing various aspects of university life.

Employment: Barriers exist both pre- and post-employment, varying by geographic location and type of impairment. These include inaccessible job portals, physical infrastructure barriers, and attitudinal challenges.

Transport and Buildings: Inaccessible transport and buildings significantly limit freedom of movement and participation in society. The report highlights the need for consistent, permanent accessibility measures.

Healthcare: The study found that persons with disabilities face significant barriers in accessing healthcare, often rooted in how the medical establishment perceives disability. The induction of doctors with disabilities in the medical establishment could be a powerful way to reduce the pathologising of disability.

Sports: A survey of sports facilities in eight states found significant gaps in accessibility, including lack of accessible restrooms, sensory rooms, and communication facilities.

Political Participation: While efforts have been made to make polling stations accessible, the temporary nature of these accommodations is criticised.

The report also highlights the need to consider accessibility for less commonly recognised disabilities and to take note of intersectional factors like gender and location. It includes testimonies from individuals with autism, cerebral palsy, deaf-blindness, intellectual disability, psychosocial disability, sickle-cell disease and the rare condition of ichthyosis.

Key recommendations include:

- Striking down Rule 15 of the RPD Rules since it is *ultra vires* the RPDA.
- Redrafting accessibility rules clearly specifying the mandatory minimum standards.
- Interim court guidelines to prevent regression of accessibility rights.
- Mandatory observance of existing technical standards (e.g. National Building Code, Indian Roads Congress guidelines, Guidelines for Indian Government Websites).
- Specific accessibility measures across disabilities (e.g. low-floor buses, multi-modal communication).
- Distinguishing between immediately mandatory accessibility requirements and those that can be progressively realised.

The report calls for a more robust, enforceable framework to create a truly inclusive society. It stresses that without addressing these fundamental issues of accessibility, many rights guaranteed to persons with disabilities would remain unrealised. The report urges immediate action to rectify the legal inconsistencies and implement practical measures to ensure accessibility across all sectors of society. “Finding Sizes for All” reflects the need to consider accessibility not just in physical spaces but also in the way services, information, and opportunities are delivered—ensuring that sizes are created to fit the needs of disabled persons and that their requirements are not sized down to fit the demands of others.

I. Introduction

- 1.1 When the construction of the material world, the organisation of services, the establishment of means of communication and transport are planned and executed without taking the requirements of persons with disabilities into account, such planning and execution invariably results in their exclusion. Before the emergence of disability rights, the world was unquestionably constituted oblivious of the existence of persons with disabilities. To reverse this process, there was firstly a move towards universal design, which entails adopting a design which allows for maximal inclusion but not the inclusion of all. Inclusive design, on the other hand, is to envision the world in such a way that no one is left behind. This report—documenting the accessibility initiatives of the country—is an effort to look at Indian accessibility law and policy from the *inclusive* lens of disability rights.
- 1.2 Accessibility is the first requirement in the creation of a disability-inclusive world. Without ensuring accessibility, all other rights guaranteed to persons with disabilities become meaningless. Making the world disability-inclusive requires two kinds of initiatives: one, which rectifies the errors of the past and the other, which plans for the future. The future plans are required in order to ensure that the errors of the past are not repeated. This means that all future creation of the world must be disability-inclusive. Rectifying the errors of the past entails renovating and rebuilding the erstwhile inaccessible built environment and making it accessible. Once the injustice of disability exclusion is accepted, all future developmental activity has to be undertaken in a disability-inclusive manner. This obligation is to be performed *immediately*. On the other hand, the task of reconfiguring the inaccessible world and removing the obstacles to accessibility has to be undertaken in a staggered but time-bound manner. It is important that the strategies of both kinds of activities are kept distinct and separate, and the permission of progressive realisation attaching to the obligation of reconfiguration is not extended to *developing* an accessible world. The Accessibility Guidelines of

1998 as also the Harmonised Guidelines of 2016 and 2021 (hereinafter HG 16 and HG 21, respectively) are tools to build an accessible world, whereas the Accessibility India Campaign and other such initiatives are attempts to recreate the country's infrastructure.

- 1.3 The adoption of the Convention on the Rights of Persons with Disabilities (hereinafter CRPD) by the United Nations is accepted as one of the most significant developments in the evolution of the rights of persons with disabilities. Along with equality-non-discrimination and awareness raising, the right to accessibility has been explicitly incorporated as a cross-cutting right by the CRPD. In the earlier drafts of the Convention, clauses relating to awareness raising and accessibility needed to be incorporated in every right recognised by the CRPD. This necessity of reiteration brought home the cross-cutting nature of the right—which is now a general principle of the Convention under Article 3—and how without removing the mental and physical barriers which impede its realisation, disability rights cannot become a lived reality. The need to fulfil this obligation assumed by the country in signing and ratifying the CRPD was acknowledged in the provisions incorporated in the Rights of Persons with Disabilities Act, 2016 (hereinafter RPDA). The RPDA—in comparison to the Persons with Disabilities (Equal Opportunity, Protection of Rights and Full Participation) Act, 1995 (hereinafter PWDA)—has defined disability in expansive terms by including as many as 21 impairments and retaining the possibility of adding more impairments to this list. The RPDA is the operable law in the entire country, under which accessibility obligations have to be fulfilled in relation to all persons with all disabilities in all parts of the country. It is due to this foundational quality of the right that disabled people and their organisations have been advocating and agitating for its realisation. *Rajive Raturi vs Union of India*,¹ the social action litigation filed in the Supreme Court, was an effort to seek the support of the Court towards this end.

¹ WP (Civil) 243/ 2005.

- 1.4 The concern of the petitioner was taken on board by the Court and, in order to allow for informed intervention, notice was issued to the Union, States and Union Territories to file comprehensive affidavits on their implementation of the right to accessibility. While some affidavits were filed before the Court, the reporting was found to be inadequate and incomplete. It was in these circumstances that the Court, by its order dated November 29, 2023, designated the Centre for Disability Studies, NALSAR, Hyderabad (hereinafter CDS) to inquire into the matter of accessibility rights in the country and submit a comprehensive report to the Court. CDS was asked to coordinate with the Union Ministry of Social Justice and Empowerment, Government of India, for logistical support.
- 1.5 CDS, in its response, requested the Court to permit it to adopt such methodology as it deems appropriate, including roping in law students, disabled people and their organisations and consult with accessibility experts, in order to prepare a status report on accessibility rights in the country within a period of six months. The Court by its order dated January 13, 2024, allowed CDS's request and accorded it the freedom to devise appropriate procedures to create a Status Report on the Right to Accessibility in India.
- 1.6 The Supreme Court in several judgments² has stressed on the diversity of disability and how one size does not fit all. The Court has also asked that due consideration be accorded to the fact of intersectionality and how disability intersects with other vectors of disadvantage and vulnerability such as caste, sex, region and religion. In framing its terms of reference and organising the structure of this report, CDS has been guided by these pronouncements of the Court.

² *Vikas Kumar vs UPSC* (2021) 5 SCC 370; *Patan Jamal Valli vs State of Andhra Pradesh* 2021 SCC Online SC 343; *Syed Bashiruddin Qadri vs Nazir Ahmed Shah* (2010) 3 SCC 603.

1.7 While examining the right to accessibility CDS has firstly distinguished between:

- a) Ensuring accessibility in the execution of all ongoing and new activities, be it in relation to built environment, transportation, communication or services;
- b) Transformation of already built infrastructure in order to make it accessible.

1.8 Since the CRPD and the RPDA have guaranteed accessibility to all persons with disabilities the report has attempted to find out:

- a) Whether the accessibility norms formulated for the country have taken note of the geographical, social and economic diversity in India?
- b) Accessibility is commonly viewed as a concern of persons with blindness, locomotor disability and hearing impairment. Whereas denial of accessibility is equally compromising for other disabilities—for example, the need for plain language for persons with intellectual disabilities and quiet places for persons with autism. How have these concerns been addressed in law and policy?
- c) As already stated, accessibility is the foundational layer of all rights. Without accessibility, other rights remain paper rights. Hence, in its third section, this report looks at the impact of the denial of accessibility on other rights. The report focuses on: access to justice, custodial justice, education (especially in government schools), right to employment, freedom of movement through various means of transportation, right to health, and the right to sport.

1.9 To execute the terms of reference CDS has employed the following methodology:

- a) CDS wrote to the Union government, State governments, Union

Territories, court administrations and prison establishments seeking details of both prongs of their accessibility initiatives.

- b) Since the access needs of all impairments are not taken on board, the accessibility needs of the neglected impairments have been documented by obtaining affidavits from persons with the specified disabilities. Clause 6 of the RPDA Schedule allows for further inclusions. To enable such initiatives, the testimonies of persons with impairments not covered by the statute have also been obtained and included.
- c) It is undeniable that persons with disabilities have the highest stake in the efficient implementation of the right to accessibility, even though, in the long run, an inclusive polity would benefit all. The Court's mandate to CDS was, therefore, widely shared and disabled people and their organisations were invited to feed into the report, in recognition of their expertise and experience and **so that the Court would have the benefit of hearing the submissions of the state and the people**. The situation on the ground in relation to particular rights across the country was ascertained by a network of organisations by conducting surveys on the right in question. How access impacts on the right to education in government schools, how transport services impact on the right to movement and how the right to sport is organised on the ground are some of the questions on which the report has been formulated with inputs from people's research. Law students participated in the access to justice reports and undertook accessibility audits in Mumbai.
- d) Accessibility has increasingly come to be seen as a right driven by technical expertise. Often, initiatives to realise the right become disputes between experts. In order to understand the impact of the Harmonised Guidelines and to obtain inputs on how to improve the observance of the guidelines, interviews were conducted with experts on access. Since doctors with disabilities could be empathetic interlocutors for the health rights of persons with disabilities, a focused group discussion was held

with doctors with disabilities. An expert written opinion was obtained on the manner in which the right to higher education has been complicated by matters of access. A first-person testimony from a poet-author with disability was obtained in order to bring on board the right to home, family and relationships.

- e) The CRPD requires all disability-related deliberations to be informed by the views of disabled people and their organisations. This concern has been kept primary in the methodology adopted in the formulation of this report. Towards this end and to ensure accountability, a draft of the report was first shared for comments and feedback with all the individuals and organisations who participated in its making, and their contributions have been acknowledged in this report.
 - f) The Court, in its January 13 order, had permitted CDS to incur expenses for the report, informed by the values of public frugality, and seek reimbursement of the same from the Union Ministry of Social Justice and Empowerment. We think we should inform the Court that this report has been made by each of us using our own resources, and no financial claims have been made to the Ministry of Social Justice and Empowerment.
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II. Realising Accessibility: The Responses of the Union, States and Union Territories

- 2.1 In order to document how the union and state governments have acted to realise the right to accessibility CDS sent mails seeking data from 28 States and 8 Union Territories. **Table II.1 (Annexure I)** shows the communications sent. The table records the date of the first communication and subsequent reminders, as also the date on which the States responded to our request. The table shows that whilst CDS sent its first request in February 2024 to all States and Union Territories, we only heard from **14 States and 3 Union Territories**. Even amongst the governments that responded, some responses (including the one from the Union Ministry of Social Justice and Empowerment) came as late as June 27, 2024. **14 States and 5 Union Territories did not respond.**
- 2.2 **Table II.2 (Annexure I)** has disaggregated the data provided by the States in order to understand how different States have undertaken to fulfil their accessibility obligations. It is found that the States have primarily concentrated on their retrofitting responsibilities. Thus, the affidavits recount the State's progress in making inaccessible buildings accessible. In order to retrofit a building, the State needs to identify a building, get an access audit undertaken, decide how the building should be retrofitted and then implement the accessibility plan. Arunachal Pradesh, Assam, Bihar, Chhattisgarh, Jharkhand, Haryana, Kerala, Meghalaya, Mizoram, Nagaland, Punjab, West Bengal, Andaman and Nicobar and Chandigarh have provided lists of the buildings that have been retrofitted by the respective States and Union Territories. **Table II.3 (Annexure I)** shows which kinds of buildings have been accorded priority by the States in executing their retrofitting plans. The States have primarily accorded priority to schools, hospitals and panchayat raj institutions. Occasionally, a juvenile home or a stadium have made the cut.

- 2.3 The tasks of retrofitting were not completed by all States, a number of states namely Delhi, Odisha and Telangana only reported undertaking the preparatory exercises of conducting access audits or conducting access training or identifying buildings which needed to be made accessible. It is to be noted that all the activities are reported by the States as *doing accessibility* and no distinction is drawn between preparing for accessibility and implementing accessibility measures.
- 2.4 As already mentioned in Part I of this report, accessibility encompasses both planning for the future and rectifying the past. However, the reports of the States ignore the former and refer only to the extent to which they have undertaken the task of retrofitting, be it in relation to the built environment, transport or websites. In the context of retrofitting, Section 45(1) of the RPDA stated that all existing public buildings shall be made accessible in accordance with the Rules formulated by the central government within a period *not exceeding five years* from the notification of the Rules. The proviso to the section permits the Union Government to extend this deadline for particular States on a case-to-case basis. Neither the Union nor the States (and Union Territories) acknowledge that the statutory time limit has been crossed. They also make no mention of any deadline by which they plan to meet their statutory obligations.
- 2.5 The Affidavits of the Social Justice Ministry only report on the buildings that have been made accessible. There is no information on how many buildings are still pending and by when the government expects to complete the exercise. Whilst the retrofitting duties in relation to websites has been reported, no information has been provided on the actions taken to make for accessible transport.
- 2.6 As already mentioned, the Union Ministry and the States and Union Territories only report on its retrofitting activities. There is no report on how the State is ensuring that the norms of accessibility are being observed in relation to all new activities of building, communications, transport or service

provision. Section 40 of the RPDA states that the standards of accessibility would be notified in the Rules by the Central Government in consultation with the Chief Commissioner of Disability.

2.7 However, Rule 15 of the RPD Rules does not formulate rule-like norms, it only specifies guidelines on building activity, transport, websites and the chosen sectors—be it rural areas or railways, ports or education. The basic distinction between Rules and Guidelines is that whilst the observance of Rules is non-negotiable, there is flexibility in the observance of Guidelines. The need for such flexibility in the realm of accessibility is expressly acknowledged in HG 2021. This flexibility is needed to accommodate the geographical and cultural diversity of the country as also the diversity of disability. If reaching the outcome of inclusion requires that the adoption of a one size way of doing things should not be insisted upon, then it was important to distinguish between Rules of Accessibility and Accessibility Guidelines. The consequence of collapsing this distinction shall be considered in detail in the last Part of this report.

2.8 Section 44 of the RPDA makes observance of the accessibility norms mandatory. Section 44(1) states that “(n)o establishment shall be granted permission to build any structure if the building plan does not adhere to the Rules formulated by the central government under section 40.” And section 44(2) declares that “(n)o establishment shall be issued a certificate of completion or allowed to take occupation of a building unless it has adhered to the rules formulated by the central government.”

2.9 However, Rule 15(1) of the RPD Rules requires every establishment to comply with the accessibility standards relating to physical environment, transport, and information and communication technology expressly mentioned in the Rules. These being:

- a) The Harmonised Guidelines and Space Standards for Barrier Free Built Environment for Persons with Disabilities and Elderly Persons issued by

the Ministry of Urban Development in March 2016

- b) Standard for Bus Body Code for transportation System specified by the Ministry of Road Transport in March 2016
- c) The website standards as specified in the guidelines for Indian Government websites
- d) The documents on the websites were to be in accordance with the electoral publication or optical character reading pdf format.

2.10 The proviso to Rule 15 committed to notify standards for other services and facilities within six months of the notification of the Rules. In 2023 the Harmonised Guidelines 2016 were substituted by the 2021 Guidelines and this change was notified in March 2023. Also, the Ministry of Social Justice and Empowerment added guidelines with regard to rural areas, water management, toilet design, police, prisons and disasters, ports, and education. Even as the incorporation of these guidelines in Rule 15 has been notified by the Ministry of Social Justice and Empowerment, the guidelines were formulated by the Ministry that is in charge of the domain. Illustratively, the guidelines on Railways have come from the Ministry of Railways and the accessibility norms for rural areas have come from the Ministry of Rural Development.

2.11 Rule 15 (2) places the responsibility of ensuring compliance of the standards of accessibility specified under the Rule on the respective ministries and departments, either by an existing domain regulator or a regulatory body created for this purpose.

2.12 There is a difference in the manner in which compliance has to be obtained for rules and for guidelines. The consequences of disobeying guidelines cannot be the same as disobeying rules. In fact, punishing the non-observance of guidelines the same as rules could be challenged as breaching

the norms of natural justice. Yet, legal compulsions and sanctions are needed to ensure that the Accessibility Rules are complied with.

Interviewing Access Experts on the Accessibility Norms and Procedures

2.13 In order to understand the way out of this conundrum we interviewed some experts who have been looking closely at the ground-level access situation in the country and whose expertise is informed by their experience. Many of them stated that whilst guidelines could grow and evolve keeping in harmony with developments in knowledge and technology, Accessibility Rules should incorporate the non-negotiables and not be altered too quickly. Since awareness-raising and training is key to ensuring accessibility, a constant change of rules only causes confusion. This results in ambiguity and uncertainty becoming excuses for non-observance. People offer a lack of clarity in the rules as their reason for not following them. If accessibility is to be embraced by the people, then they must understand it and not be overwhelmed by it. The objective of accessibility—that is, inclusion—should be understood by all and a commitment to remove the most evident barriers spelled out. The law should place a duty of ensuring accessibility on all and the guidelines should suggest the various ways by which the goal can be realised. An overly detailed, uni-dimensional mandate may prove to be counter-productive.

2.14 The access experts pointed out how the effort of the government seems to be to realise mandatory intent of the statute through voluntary Good Samaritan activity. Instead, what was needed was to make an accessibility clearance an integral part of obtaining a completion certificate for buildings or registering a website or launching a transport service. In order to ensure that the accessibility facilities do not fall into disuse, a renewal of the disability clearance certificate was also suggested. A system analogous to obtaining a

fire-safety certificate could be adopted to ensure the due observance of accessibility norms.

2.15 Rule 15 has as many as 14 sets of fully notified Guidelines and two draft guidelines. Experts were divided on this strategy employed by the Ministry of Social Justice. One body of opinion was that the demands of accessibility altered depending on the domain. And the demands of the domain were best expressed by subject experts. Therefore, to obtain the most suitable guidelines on the domain, it was best to seek it from domain experts. Another set of experts challenged this opinion pointing out how this outlook does not recognise the experiential and educational expertise of disability experts. Thus, for example 4.4.2 of the Ministry of Rural Development Guidelines requires a ramp to be constructed for each house allotted to a person with disability, suggesting the ramp be constructed by the beneficiary themselves from the assistance provided to them. The guideline presumes all persons with disabilities require a ramp and has placed the financial burden of constructing the ramp on the person with disability. It is expected such errors would not arise from government officials working in the field of disability.

2.16 Sub-section (2) of section 40 has placed the responsibility of obtaining compliance on the concerned ministry or department. The Ministry of Rural Development has in Guideline 5.4 placed the obligation of implementing and monitoring the guidelines on Village Level Committees to be formed by Gram Panchayats. Whilst the experts welcomed the decentralisation, they questioned the effectiveness of such monitoring, as the monitoring body has not been given a role in the formulation of the Guidelines. Concerns were expressed on the large spread of the regulatory establishment.

2.17 Three kinds of suggestions were provided to streamline the regulatory situation:

- a) India could look at the US Model of 3 Codes: (a) Physical Environment which is regulated by the ADA Code and overseen by the Department of

Justice; (b) Transport Code which includes all forms of transport, overseen by the Transport Department and (c) Virtual Accessibility Code, regulated by section 508 of the Rehabilitation Act. All three Codes are driven by the principles of Universal Design and provide templates of various case scenarios as annexures.

- b) Another suggestion was to adopt the harmonised UN ESCAP standards as the national standard after vetting by technical experts and persons with disabilities.
- c) A third was to identify and incorporate accessibility requirements into relevant existing regulations, building codes, and approval processes for new constructions and occupancy certificates. Designate a specific authority, such as the Disability Commissioner, to receive periodic accessibility compliance reports from establishments and issue certifications based on third-party audits by empanelled access auditors. And to ensure periodic re-audits and renewal of accessibility certifications, to address maintenance issues.

2.18 These suggestions on improving enforceability as also the import of the distinction between rules and guidelines shall be revisited in Part V of this Report.

III. Accessibility for All Impairments

- 3.1 In a significant and welcome change of perspective from the Harmonising Guidelines 2016, the Harmonising Guidelines 2021 introduced a chapter on “Accessibility, Diversity and Universal Design”. With this chapter, the Guidelines acknowledged that demands of accessibility are not limited to persons with visual, hearing and locomotor impairments. All persons with disabilities have accessibility demands, and a truly inclusive environment needs to address them all. More importantly, the 2021 Guidelines acknowledge that accessibility needs are not limited to persons with disabilities alone. Children, the elderly, pregnant women, others who may be temporarily disabled have accessibility needs as well. Accessibility needs also alter depending upon a range of social, economic and geographical factors. In the wake of this wide-ranging diversity, the Guidelines advocate flexibility and provide a range of options, which could be tailored according to locale, socio-economic factors and nature of impairment. It then undertakes an exposition of different kinds of impairments and makes a connection between the impairment and its accessibility needs. With this chapter HG 2021 has provided a more encompassing outlook towards accessibility.
- 3.2 Like policy, the law adopts a more liberal outlook towards disability. Whereas the PWDA had restricted its operation to the seven named impairments, the RPDA has spread its net wider. The Act recognises that irrespective of percentages, a person with a lesser degree of impairment could still experience comparable discrimination. Thus, the RPDA provides protection against discrimination to all persons with disabilities. The Supreme Court has unequivocally ruled that reasonable accommodation be provided to all disabled persons and not only to those with benchmark disability.³ The RPDA, however also acknowledges that persons with a higher degree of impairment need greater support. Such support may be provided through

³ *Vikas Kumar vs UPSC* (2021) 5 SCC 370

job reservations for persons with benchmark disability and special schemes for those with high support needs. Since generic definitions on disability have been found practically non-feasible, the RPDA has named 21 new impairments and retained scope for the addition of more impairments.

3.3 In spite of acknowledging a connection between accessibility and diversity, the HG 2021 has only sketchily dwelled on the accessibility needs of less mainstream disabilities. The lack of access significantly impacts the rights of persons with these other impairments. In order to foreground the accessibility needs of persons with less-discussed impairments, CDS has obtained affidavits from persons with autism, cerebral palsy, deaf-blindness, intellectual disability, psychosocial disability, sickle-cell disease and ichthyosis—a rare skin disorder which has not as yet been included in the RPDA though persons with the condition are subjected to unparalleled discrimination.

3.4 These affidavits outline the particular accessibility needs of persons with the above-mentioned conditions. Thus, persons with autism speak of the advantages of a communication board and how subsidised electronic devices could make communication easier for persons with autism, cerebral palsy and deaf-blindness. The need to create a register of scribes is the other accessibility demand put forth by persons with cerebral palsy. The testimony of persons with intellectual disability has been included as a third person narrative, as the interlocutors realised too late that communication strategies relating to cognitive faculties could not establish connection with their inner world. Plain language and narrative techniques had not just to be demanded from the outside world but also needed to be internally practiced. The need for plain language and pictorial representation came from both persons with autism and intellectual disability. Persons with psychosocial disabilities seek normative accommodations like relaxation in attendance, examination time and conduct of written instead of oral interviews. Persons with sickle cell-disease seek improved access to water and food due to their chronic medical

condition in addition to enhanced health support. And a person with ichthyosis testified that she “needs creams, emollients, bandages, gauze, band-aids, earbuds, socks and gloves” to cover herself from head to toe to prevent her skin from drying.

- 3.5 It is an oft stated truism that the so-called non-disabled are only temporarily able. Or, said another way, the non-disabled and the disabled are on a continuum and not in a dichotomous relationship. This is most sharply brought out by people who acquire disabilities and thereby experience a change of identity. Often this change of identity is permanent and people are compelled to embrace their new life and persona. However, the person whose testimony we have included is a classic case of temporary disability. This person met with a serious accident causing him to have more than 60% impairment. Subsequently, with intense physiotherapy he successfully regained the strength of his limbs and the percentage of disability reduced. The travails of rigid identities and the possibilities of building connection and solidarities is recounted in their testimony. This affidavit provides a strong experiential argument for universal design.

To access the affidavits, [click here](#).

- 3.6 In order to obtain a first-person version of the lives of persons with disabilities, the National Centre for Promotion of Employment for Disabled People undertook a pan-India survey to document the challenges that persons with disabilities encountered in their day-to-day life with regard to inclusivity and accessibility. The survey covered 227 persons and received responses from all 28 States though most responses were received from Uttar Pradesh, Jharkhand and Maharashtra. The report also recorded how locational disadvantages like living in a hilly area reduced accessibility.

To access the NCPEDP Report, [click here](#).

3.7 Nandini Ghosh, one of the leading experts on gender and disability, has in her exposition asked for special attention to be accorded to the intersection between gender and disability.

To read the analysis by Nandini Ghosh, [click here](#).

IV. Accessibility—A Cross-Cutting Right

- 4.1 In Part I of this report, it was expressly stated that the right to accessibility is both a right in itself and a right to have rights. General Comment 2, on the CRPD points out how the right to accessibility is a precondition to the realisation of all other rights. While Parts II and III of the report looked at the right to accessibility per se, in this Part, the intersectional nature of the right shall be explored. The objective is to demonstrate the impact of the right to accessibility on rights in general.
- 4.2 This section has primarily been navigated through pilot surveys, research studies, expert expositions and personal testimonies from subject and experiential experts.

Accessibility in Courts

- 4.3 There is no right without a remedy is an oft stated legal credo. Insofar as courts are often the site to devise this remedy, CDS requested Ms Roma Bhagat and Associates to undertake a pilot survey of all the High Courts in the country and report where they stand in relation to accessibility for litigants and lawyers with disabilities. No response was received from the High Court of Allahabad, the High Court of Allahabad (Lucknow Bench), the High Court of Madhya Pradesh (all benches) and the High Court of Jammu and Kashmir and Ladakh (Srinagar). The response from the Madras High Court (Madurai Bench) was received after the analysis of the data was complete. The data on the Delhi High Court has not been validated by the High Court. The responses from 30 High Court benches have been analysed both quantitatively and qualitatively, with the qualitative analysis questioning the quantitative analysis made by the High Courts and necessitating a physical inspection and confirmation. The survey found that the accessibility needs for persons with locomotor disability were better addressed than what was provided to other disabilities. Tactile guiding strips were provided by only 2 out of 30 High Court benches. Similarly, interpreters

for the hearing impaired were only available in 2 Courts. There was only 6.6% awareness of the needs of persons with intellectual and developmental disabilities.

To access the detailed report in relation to all the High Courts, [click here](#).

4.4 To understand the complexity of the ground situation, Mr Rajesh Deoli undertook a preliminary survey of District and Sessions Courts at Rudraprayag and Chamoli and the Sub-District Court at Karanprayag in Uttarakhand. The Courts are not easy to reach due to lack of bus service and in the Court building the Legal Services Authority has been situated on the first floor without a ramp or lift. A toilet has been designated for persons with disabilities but cannot be used.

To access the full report, [click here](#).

4.5 In contrast to these two studies which document the present situation in the courts they surveyed, Mission Accessibility has provided a blueprint on how the functioning of courts need to alter in order to make the court system accessible.

To access the Mission Accessibility report, [click here](#).

Accessibility in Prisons

4.6 To ensure that persons with disabilities are only sent to prisons as punishment and not for punishment, a questionnaire was sent to all the prison administrations in the country. On the basis of the responses received it was found that:

- a) Most states claim to provide wheelchairs/crutches, western toilets, and prosthetics.
- b) They also claim they provide medical referrals to civil hospitals.
- c) Some states provide special diets and counselling services.

- d) Facilities are mostly available in central prisons and special prisons for women, with limited availability in district and sub-jails.

4.7 The report identified the following systemic issues:

- a) Lack of Recognition: Many states don't formally recognise all categories of disabilities as defined by the Rights of Persons with Disabilities Act, 2016. This lack of recognition likely leads to underreporting and inadequate provision of services.
- b) Concentration of Services: Better facilities and services tend to be available only in central prisons and special prisons for women, leaving prisoners in district and sub-jails underserved.
- c) Inadequate Staffing: There's a severe shortage of mental health professionals in prisons. The report indicates that for the current prison population, there should be at least 1,146 psychologists/psychiatrists, but only 69 posts are sanctioned, with just 25 filled.

To access the complete report on accessibility issues in prisons, [click here](#).

Accessibility in Educational Institutions

4.8 Even with legal provisions in place, children with disabilities face numerous challenges in accessing their right to education. To gauge the current status of access to education for children with disabilities, 947 parents of children with disabilities attending government schools were surveyed through a collective effort of 41 CSOs and individuals across 23 Indian states.

4.9 Fifty-six percent (56%) of these children studied in primary classes with a sharp drop in enrolment in higher grades. Four percent (4%) of the students never went to school even though enrolled, some because schools asked them not to come and some because of inaccessible transport. Infrastructural accessibility forms the threshold for holistic inclusion of children with disabilities in society, including educational institutions. In

some schools, the classrooms were not accessible and the parents were asked to ensure the child reached the class. Only 20 students (2%) could access the toilets. They could not access the assembly and mid-day meals. Only seventeen percent (17%) of students were provided learning material by the school and sixty-four percent (64%) of the students could not study because the formats available were not accessible to them. Being present in classrooms and yet feeling invisible and forgotten is a humiliating experience for children with disabilities (both visible and invisible).

For the full report on accessibility in government schools, [click here](#).

4.10 In addition to the study led by Astha, the impact of access on educational rights of persons with disabilities was also documented in the NCPEDP study.

To access the NCPEDP study, [click here](#).

4.11 The status paper by Prof. Sudesh Mukhopadhyay helps in comprehending the multiple ways in which access influences higher education.

To access the status paper, [click here](#).

Accessibility in Disaster Management

4.12 The need for dovetailing accessibility concerns in disaster management strategies has been argued for by both Dr Asha Hans and Dr Viswesh Sekhar.

To access the advocacy notes and a peoples' strategy to cope with disasters, [click here](#).

Accessibility in Employment

4.13 We received a report from Enable India which describes the issues faced by persons with disabilities, persons with benchmark disabilities and persons with disabilities having high support needs, in the context of employment owing to lack of accessibility. These barriers exist both pre- and post-

employment and are also connected with geographical location and nature of impairments.

Table IV.1

<i>Geographic Context</i>	<i>Primary Barriers</i>	<i>Examples/Inferences</i>
<i>Urban Areas</i>	Inaccessible job portals, complex application processes	Difficulty in navigating online job applications
<i>Rural Areas</i>	Lack of accessible educational and vocational facilities	Limited access to skill development programmes
<i>Tier 2 Cities</i>	Social integration challenges	Limited exposure affecting job readiness
<i>Linguistically Diverse Regions</i>	Information Accessibility	Difficulty in accessing employment information in local languages
<i>Pan India</i>	Inconsistent implementation of accessibility standards	Varying levels of infrastructural accessibility across regions

Table IV.2

<i>Type of Impairment</i>	<i>Primary Barriers</i>	<i>Examples/Recommendations</i>
<i>Visual Impairments</i>	Inaccessible digital platforms, lack of accessible course materials	Need for screen-reader compatible job portals, alternative assessment methods

<i>Type of Impairment</i>	<i>Primary Barriers</i>	<i>Examples/Recommendations</i>
<i>Hearing and Speech Impairments</i>	Lack of sign language recognition, inaccessible application processes	Constitutional recognition of sign language, inclusive communication methods in applications
<i>Locomotor Disabilities</i>	Physical infrastructure barriers	Ensuring compliance with accessibility standards in buildings
<i>Neurodevelopmental Disorders and Learning Disabilities</i>	Rigid selection criteria, inaccessible learning environments	Use of practical tests instead of aptitude tests, universal design learning framework
<i>Multiple Disabilities</i>	Inaccessible recruitment examination systems	Need for inclusive assessment methods to ensure 1% reservation is honoured
<i>Acid Attack Survivors</i>	Attitudinal barriers, lack of awareness	Sensitisation and awareness building about capabilities

To access the report, [click here](#).

4.14 The NCPEDP pan-India survey has also recorded the employment status of persons with disabilities. The survey found persons with locomotor disability and visual impairment with higher employment than persons with other impairments.

For more information on employment across disabilities, [click here](#).

Accessibility in Buildings and Transport to Ensure Freedom of Movement

4.15 Freedom of movement is a right that relies on both the ability to move freely (freedom of transport) and the ability to access destinations (right to enter buildings). Restrictions on either of these aspects can effectively limit an individual's overall freedom of movement and participation in society.

4.16 Accessible transport is crucial for realising the rights of disabled persons. It becomes a gateway to education, employment, healthcare, and social participation. For instance, in Delhi, 3,775 low-floor CNG buses enable wheelchair users to access education and employment independently. Whereas, Tamil Nadu reports only 1,917 out of 21,669 buses are accessible. When transport is inclusive, it makes essential services accessible. Thus, for instance, the installation of effective audio systems in buses significantly improved the transaction of everyday activities such as keeping health care appointments for visually-impaired participants. On the other hand, inaccessible infrastructure, like pothole-ridden footpaths causing injuries, directly impedes rights to safety and mobility.

For the detailed report on accessibility in transport, [click here](#).

4.17 Mandatory building compliance for accessibility is crucial for realising disabled persons' rights to education, employment, and healthcare. In order to enable understanding of the connection between accessible buildings and other human rights a study was undertaken by Dr Suman Kalani of SVKM Pravin Gandhi College of Law, Mumbai along with her students for this report. This study evaluating the accessibility of buildings in Mumbai found a mixed picture. Newer infrastructure like Andheri Metro Station met the demands of accessibility with special entrances and lifts for disabled individuals. Older buildings like the Bombay Art Gallery lack basic features such as accessible restrooms. This disparity significantly impacts freedom of movement. For instance, the accessible Andheri Metro Station enables disabled persons to commute independently, enhancing job opportunities. Conversely, the

inaccessible main entrance of Krishna Curve Shopping Mall at Santa Cruz denies persons with disabilities the opportunity to either give business or seek employment. The survey found the current regulatory framework to be fragmented which weakens the realisation of accessibility.

To access this study on accessibility of buildings in Mumbai, [click here](#).

Accessibility and the Freedom to Form Relationships

4.18 In settling up the accessibility priorities, emphasis is accorded to education, employment and health. Emotional and relational needs are ignored. Abhishek Annica, poet-author and disability activist, points out how “(d)isabled life can be quite lonely and alienating, often by design. All questions are segregated into the public and the private. In the discourse on accessibility, the public questions are encouraged, the private ones are made to look trivial. Questions about access to desire. Questions about access to love. And what about sex. ... I come across many disabled individuals struggling with questions of desire. ... even self-pleasure is impossible without assistance... (since) most disabled people live with their families ... there is no access to private space.”

To read Abhishek Annica’s testimony on this vital issue, [click here](#).

Accessibility and the Right to Health

4.19 The online panel discussion organised by Dr Satendra Singh of Doctors with Disabilities: Agents of Change for this report emphasised that the biggest barrier that persons with disabilities face in accessing health care and treatment relates to how the medical establishment perceives their bodies and minds. Insofar as the disabled body and mind is seen as an aberration, the medical establishment views their condition as per se pathological. This pathologising of their condition is substantially reduced when doctors with disability are part of the treating team and the medical establishment. However, the obstacles faced by persons with disabilities to become doctors

and then to obtain the reasonable accommodation they need to function effectively are significant. The personal testimony of Dr Dhruv Pathak outlines the barriers faced by all doctors especially the ones who are neurodivergent.

To access a more detailed exposition of the [panel discussion](#) and Dr Pathak's opinion, [click here](#).

4.20 In addition to the panel discussion the NCPEDP undertook a survey which documents the challenges persons with disabilities encounter in their day-to-day life in accessing health care.

The findings of the NCPEDP survey can be obtained [here](#).

4.21 A larger number of barriers faced by persons with disabilities in the field of health occur because the non-disabled world does not ascribe value to a disabled life. This prejudice was documented in some of the studies carried out by Sangath during the pandemic on the health care travails of persons with disabilities.

To access the studies conducted by Sangath, [click here](#).

Accessibility and the Right to Sports

4.22 To understand how accessibility influences the realisation of this right, reliance is placed on a survey conducted for this report by Vidyasagar along with 14 other organisations and undertook a survey of sports facilities in eight states namely Andhra Pradesh, Assam, Ladakh, Odisha, Nagaland, Meghalaya, Jharkhand, and Gujarat. In addition, a detailed accessibility survey was undertaken by Rajiv Rajan of both the indoor and outdoor facilities of Jawaharlal Stadium, Mayor Radhakrishnan Stadium and Velachery Aquatic Center located in Chennai.

4.23 The study found only 21.4% of surveyed locations had fully accessible restrooms, with issues ranging from narrow doorways to lack of grab bars and

high washbasins. The absence of this basic facility severely impacts athletes' ability to prepare for training or competition. The absence of sensory rooms (available in only 3.6% of locations) and the lack of assistive listening systems (absent in 89.3% of venues) create additional barriers for athletes and spectators with sensory processing disorders or hearing impairments. Communication accessibility is another significant issue. Across all surveyed venues, staff lacked basic training in sign language, and a notable absence of alternative communication modes. Only 7.1% of locations had even partially functional facilities for captioning and sign language interpretation. Only 25% of surveyed sports complexes had fully accessible websites, while a mere 7.1% offered fully accessible apps.

To access the report, [click here](#).

Accessibility and the Right to Political Participation

4.24 Smitha Sadasivan of Vidyasagar examined the impact of absence of accessibility on the political rights of persons with disabilities. It is crucial to note the hybrid nature of the right in question: the right to vote is fundamentally a political right, whilst accessibility is a civil right.

4.25 This intersection underscores the complexity of ensuring full participation for persons with disabilities in the democratic process. Through concerted efforts by the civil society and the Election Commission to make polling premises accessible, disabled persons were able to exercise their right to vote, thus enhancing their political participation. However, it was found that the Election Commission has been erecting temporary ramps and facilities for every election, which are subsequently removed. Sadasivan's contention is that these accessibility facilities should be permanent to ensure consistent and reliable access for persons with disabilities.

To access the report on political rights, [click here](#).

4.26 This section of the report aimed to demonstrate with ground level data how the absence of accessible infrastructure results in a global deprivation of rights for persons with disabilities. Keeping in view these consequences, it is imperative that suitable interventions are devised towards the realisation of the right to accessibility. The next segment discusses how this could be done.

V. Findings and Recommendations

Self-Regulation is Not Enough

- 5.1 A constant refrain we heard from all quarters—be it the disability community, access experts, or domain specialists—was about the need for enforcement and third-party oversight. Self-regulation, we were told, was not enough. If deviance invited no adverse consequences, the chances of people observing the Guidelines were seen as small.

Enhance Powers of the Chief and State Commissioners

- 5.2 The other common suggestion which came from several quarters related to the office of the Chief Commissioner and Commissioners. Since these offices are present all over the country it was felt that they could be best suited to perform the oversight tasks required to enforce the rules of accessibility. This suggestion was countered by others, who pointed out that the Chief Commissioner was not powerful enough to perform these enforcement functions. To which, the response was that the Hon'ble Supreme Court could formulate guidelines suggesting to the government that S. 76 and S. 81 of RPDA be amended to make it explicit that the orders passed by Disability Commissioners were mandatory and not directory. And the Commissioners could be conferred express oversight duties in the directions issued by this Court.

Everything Cannot be Staggered

- 5.3 A third proposition that we heard from several quarters was that for accessibility to be realised all initiatives could not be progressive. A minimum floor would need to be immediately realised, whereas the increments on the floor could be progressively built.

The Relationship between the Act, Rules and Guidelines: An Analysis

- 5.4 In the face of these demands, suggestions, requests (all three tones were encountered) we were compelled to examine the relationship between the RPDA, the RPD Rules, HG 2016 and HG 2021.
- 5.5 Sections 44, 45 and 46 of RPDA have made compliance to the accessibility norms mandatory. Section 44 lays down the consequences of non-compliance, Section 45 requires retrofitting to be undertaken in accordance with “the rules on accessibility formulated by the central government” and Section 46 requires service providers to make their services accessible within a period of two years from the notification of the rules by the Central Government. A plain reading of these three sections shows that Parliament intended to use some modicum of compulsion to ensure that the right to accessibility was realised in fact. The substantive content of this compulsory component was to be incorporated in the RPD Rules which were to be finalised in consultation with the Chief Commissioner of Disability.
- 5.6 Rules for Accessibility have been included in Chapter VI of the RPD Rules. The mandatory accent has been retained in the chapeau of Rule 15, which states that “every establishment **shall** comply with the following standards relating to environment, transport, and information technology.” However, these standards which must be complied with have been incorporated in documents which are described as Guidelines.⁴ Illustratively, the standards for public buildings that the establishments are required to follow are contained in a document that is titled Harmonised Guidelines and Space Standards for Barrier Free Built Environment for Persons with Disabilities and Elderly Persons, 2016. The introductory segment to HG 2016 states: “(t)he guidelines will serve as a reference for developing future policies, guidelines,

⁴ On the non-binding nature of Guidelines see *Gulf Hotels vs Union of India* (2014) 10 SCR 536.

standards and other initiatives.” HG 2016 was substituted by HG 2021 in 2023 and these new Guidelines declare that they “aim to **sensitize, guide** and **facilitate** the approach towards implementing accessibility.”

5.7 In the text that follows both HG 2016 and HG 2021 provide choice but do not enforce compliance.

5.8 In this way, the mandatory accent of the RPDA has been transformed into self-regulation. And this change of approach has not been achieved by legislative amendment but through delegated legislation. According to the established rules of Administrative Law, the executive is obligated to exercise its rule-making powers in accordance with legislative policy. A total elimination of the compulsory dimension of the law is not in accordance with legislative policy and could be termed as *ultra vires* the RPDA.⁵ The Central Government could have formulated Guidelines **in addition** to the Rules but it could not **substitute** Guidelines for Rules as there is a material difference between these two legal artefacts. A guideline provides guidance and guidance can be accepted or rejected. Non-acceptance of guidance cannot be described as non-compliance. And without non-compliance the sanctions of Section 44 or Section 89 of RPDA cannot be enforced. **Thus, in effect, Rule 15 of the RPD Rules 2017 renders Section 44 and Section 89 of RPDA otiose.**

5.9 The accessibility provisions of RPDA demonstrated a steadfastness of purpose. The appropriate government made a commitment to realise accessibility in a timebound manner. Rule 15 has, on the other hand, taken governments off the hook and made the realisation of accessibility everybody’s responsibility, which usually translates to nobody’s responsibility.

⁵ *Supriyo vs Union of India* 2023 INSC 923; *State of Tamil Nadu vs P Krishnamurty* (2006) 3 SCR 396 (Para 12 (d)) *Union of India vs Elphinstone Spinning and Weaving Co Ltd* AIR 2001 SC 724; *Agricultural Marketing Committee vs Shalimar Chemical Works* (1997) Supp 1 SCR 164; *State of Karnataka vs H Ganesh Kamath* (1983) 2 SCR 665 at 674; *State of Tamil Nadu vs Hind Stone* AIR 1981 SC 711; *Barium Chemicals vs Company Law Board* AIR 1967 SC 295.

5.10 Before the CRPD, human rights were divided between civil-political and socio-economic rights with the accompanying jurisprudence of the former being immediately available and the latter progressively realisable. The CRPD challenged this classification and introduced the category of hybrid rights. The right to accessibility is the pre-condition for the realisation of all other rights, it also needs resources and therefore can be rightly described as a hybrid right. The minimum mandatory part of it would need to be immediately enforced, and the rest could be progressively realised. The RPDA in laying down that certain norms would be mandatorily complied with, adopted a policy which is in accordance with the CRPD. It is long established that the limitation placed on the powers of the legislature also apply on the powers of the government. The legislature has to exercise its law-making powers in conformity with international law. The same obligation has also to guide the executive in the performance of its law-making duties. **Thus, inducting Guidelines instead of Rules to execute the right to accessibility is also in breach of international law and Article 51 of the Constitution of India.**

5.11 This Report has, through first person testimonies (Part III) and several ground field surveys (Part IV), shown how denial of accessibility denies persons with disabilities the numerous other rights. The various studies in relation to the right to access to justice, education, employment, health, sports show that without at least floor level accessibility these rights become nothing more than paper rights. The audits of buildings and the various agitations around low floor buses show that just persuasion is not enough. And the above legal analysis on the relationship between the Act, the Rules and the Guidelines brings home that there is no legal procedure in place to create the minimum mandatory regime for accessibility. The Guidelines have not distinguished between the directory and the mandatory. In the absence of such distinction, all the Guidelines are directory and compliance with them has been left to the discretion of the various stakeholders. Without prescribing a minimum mandatory floor on accessibility, the government has not fulfilled its

obligations under international law. In the circumstances, it is logical to ask what is the way forward? In the last section, on the strength of our interaction with the disabled people's community as also the experts on accessibility, we make the following suggestions to the Hon'ble Supreme Court.

Suggestions on the Way Forward

- 5.12 Since accessibility is a basic right for persons with disabilities, any barrier to the realisation of this right should be removed. Rule 15 in its present form is in breach of the legislative policy of RPDA and the international obligation assumed by India in ratifying the CRPD and hence should be struck down.
- 5.13 In fulfilment of its responsibilities under the CRPD and the RPDA, the Central government would need to redraft Rule 15 of the RPD Rules. In order to make the RPD Rules in accordance with the requirements of administrative law and to be in conformity with India's international law commitments, it would be necessary that the Rules clearly spell out the mandatory obligations of the law. The minimum mandatory norms must be incorporated in the Rules. Whilst the mandatory standards must be incorporated in the Rules, there is no bar on the government to incorporate standards which are more than minimum. The government would be at liberty to provide an incentive for persons who comply with these higher standards. Since the right to participation is integral to the creation of any legal regime for persons with disabilities, the government must perform this rule-making exercise in active collaboration with people with disabilities.
- 5.14 Even as India follows the dualist system, where international law has conferred rights, this Court has relied upon the international conventions to protect the rights of especially vulnerable people.⁶ The Court has also issued guidelines in these and other cases to fill legislative vacuums, when such vacuums could leave vulnerable people without any remedy for the

⁶ *Vishaka vs State of Rajasthan* (1997) 6 SCC 241; *D K Basu vs State of West Bengal* (1997) 1 SCC 416; *Laxmi Kant Pandey vs Union of India* (1984) 2 SCC 244.

deprivation of their rights. The present situation with regard to accessibility leaves persons with disabilities in a similar situation. In the circumstances, till the government lays down the minimum mandatory component of the right, it is constitutionally imperative that this Court issues directions to ensure that there is no regression in the realisation of the accessibility rights of persons with disabilities.

5.15 One suggestion which came from many disabled peoples' organisations was the mandatory observance of the standards provided in:

- a) The National Building Code for physical environment;
- b) The Indian Roads Congress guidelines for pavements and road-based infrastructure;
- c) Guidelines for Indian Government Websites for websites and mobile applications.

5.16 These guidelines are not in themselves mandatory but could be made compulsory if the Hon'ble Court orders to that effect. HG 2021 cannot perform this job because it has not formulated any fixed standards. In fact, a number of individuals and organisations informed that some of the specifications in HG 2021 were incorrect.

5.17 The road transportation sector has some contradictory rules. The persistent demand we received from the civil society was the use of low floor buses for within-city travel. And buses with hydraulic lifts could at best ply from point to point in intercity travel.

5.18 Air travel is beset with frequent problems of inappropriate security rules (spinal cord paralysed person told to stand up by CISF security) and wheelchair inventory shortage issues. It was suggested that if the Civil Aviation Rules as on 2021 'Carriage by Air of Persons with Disability and/ or Persons with Reduced Mobility' are implemented and Special Service Requests (SSR codes) captured at time of booking, these problems would

largely disappear. The sector sought the implementation of these Rules in preference to the 2022 Rules which they describe as insulting.

5.19 All travel should ideally be with level boarding for ease of usage by all. Rail travel cannot be an exception. Increasing platform height to 920 mm, mandating wider doors and ensuring bridging ramps from trains to platforms would enable wheelchair users to travel with families instead of a separate coach. Ramped foot-over-bridge access to every platform should be prioritised before adding of elevators and escalators. For accessible last mile connectivity, adding wheelchair accessible buggies to the battery-operated car fleet will include all passengers with reduced mobility in transport terminals.

5.20 HG 2021 makes a strong case for not making accessibility a one size venture. It asks for the diversity in the polity and the sector to be embraced, so that all are included and no one is left behind. With similar motivations we have titled this report on the status of the right to accessibility in India—Finding Sizes for All.

Annexure I

Table II.1

	NAME	Request for Information from 21st February	First Reminder Sent on 4th April	Second Reminder Sent on 7th May	Third Reminder sent on 17th May	Responses
STATES						
1.	Andhra Pradesh	12th March	4th April	7th May	17th May	No Response
2.	Arunachal Pradesh	21st February	4th April	-	-	Submitted- 12th April
3.	Assam	21st February	4th April	7th May	17th May	Submitted- 22nd May
4.	Bihar	21st February	4th April	7th May	17th May	Submitted- 24th May
5.	Chattisgarh	21st February	4th April	7th May	17th May	Submitted- 6th June
6.	Goa	21st February	4th April	7th May	17th May	No Response
7.	Gujrat	21st February	4th April	7th May	17th May	No Response
8.	Haryana	21st February	4th April	7th May	17th May	Submitted - 28th June
9.	Himachal Pradesh	21st February	4th April	7th May	17th May	No Response
10.	Jharkhand	21st February	4th April	7th May	17th May	Submitted-28th June
11.	Karnataka	21st February	4th April	7th May	17th May	No Response
12.	Kerala	21st February	4th April	7th May	17th May	Submitted- 30th May
13.	Madhya Pradesh	21st February	4th April	7th May	17th May	No Response
14.	Maharashtra	21st February	4th April	7th May	17th May	No Response
15.	Manipur	21st February	4th April	7th May	17th May	No Response
16.	Meghalaya	21st February	4th April	7th May	-	Submitted- 9th May

17.	Mizoram	21st February	-	-	-	Submitted- 2nd April
18.	Nagaland	21st February	-	-	-	Submitted- 13th March
19.	Odisha	21st February	4th April	7th May	-	Submitted- 10th May
20.	Punjab	21st February	4th April	-	-	Submitted-24th April
21.	Rajasthan	21st February	4th April	7th May	17th May	No Response
22.	Sikkim	21st February	4th April	7th May	17th May	No Response
23.	Tamil Nadu	21st February	4th April	7th May	17th May	No Response
24.	Telangana	21st February	4th April	7th May	-	Submitted- 8th May
25.	Tripura	21st February	4th April	7th May	17th May	No Response
26.	Uttar Pradesh	21st February	4th April	7th May	17th May	No Response
27.	Uttarakhand	21st February	4th April	7th May	17th May	No Response
28.	West Bengal	21st February	4th April	7th May	17th May	Submitted- 24th June
UNION TERRITORIES						
1.	Andaman and Nicobar	21st February	-	-	-	Submitted-28th Feb
2.	Chandigarh	21st February	4th April	7th May	17th May	Submitted- 20th May
3.	Dadar Nagar Haveli and Daman and Diu	21st February	4th April	7th May	17th May	No Response
4.	Delhi	21st February	4th April	7th May	17th May	Submitted- 10th May
5.	Ladakh	21st February	4th April	7th May	17th May	No Response
6.	Lakshadweep	21st February	4th April	7th May	17th May	No Response
7.	Jammu and Kashmir	21st February	4th April	7th May	17th May	No Response
8.	Puducherry	21st February	4th April	7th May	17th May	No Response

Table II.2

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
CENTRE										
Ministry of Social Justice and Empowerment, Govt. of India	1723 623- states 1100- central government	-	No	-	35 International Airports 55- Domestic Airports 709- Railway stations 8695- Buses	3120- Bus stations in respect of 24 states and UTs 75- Pilgrimage places	-	-Sugamya Bharat Application is currently under update -Indian Sign Language Research and Training Centre launched 2015 -Mandatory Accessibility courses initiated in professional degrees Curriculum	672- state government 95 Central govt	There has been no updates on the measures taken for the employment of the disabled individuals.
STATES										
Arunachal Pradesh	17 government buildings		no	Yes	No	Yes	Work completed for the first	Yes	No	Proper signage and sign boards need to be fitted in designated project

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
	mainly schools and secretariat						phase of the 713.26 lacs released in the year 2023-24 which has been utilized.			areas.
Assam	20 Government Buildings	Done by Sishu Sarathi NGO 2019-20	No	No	No	No	No	No	No	Buildings identified during the 2nd phase but there is no information regarding the retrofitting and no further actions taken
Bihar	14 government buildings mainly categories not specified.	Transport-14 Education-39,802 Construction-1265 Labour-20	No	No	No	No	No	Yes	No	No specific information provided in the report.

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
Chattisgarh	42 buildings not specified properly	No	No	No	No	No	2019-20: 19,88,22,000 2020-21: 91,77,000 1,10,25,750 unutilized amounts have been refunded back to the government of India from year 2019-2020 on 22.05.2024	No	Yes Out of 93 Websites 66 were made accessible	No money has been utilized in past 3 years
Jharkhand	14 buildings mainly secretariat and government colleges and government schools.	Yes	No	Yes	2 seats reserved	Yes	11.6685 Cr- Received 7,55,41,586 Cr. - Utilized	Training of Sign Language Interpreters	7 websites	4,11,43,863 Cr remaining amount not utilized and no information is sent as to how the money will be further used.

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
Haryana	Phase 1-54 Phase 2- 2 Phase 3- 26 Not specified on categories of buildings	-	No	No	Yes	No	1544.95 lacs have been released for retrofitting.	No	Yes 76 State Government websites have been made accessible.	Out of 62 buildings 54 have been completed, the rest are still pending.
Kerala	28(Phase-1) 6(Phase-2) Mainly schools and government colleges , district panchayats, hospitals.	2019	No	Yes	No	Yes	8.59 cr	Yes	Yes 724 websites were made accessible in 2019.	Pending work on ATM accessibility and works for the vision panel must be completed. There must be strict adherence to the initiatives taken by the State for accessibility.
Meghalaya	21 buildings mainly hospitals and government offices and government schools and	Done by CBM Trust in 2016	No	No	No	Yes	Not specified	Yes	No	Last access audit was done in 2023 but no information has been provided for the same even the progress report for the access audits have not been attached.

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
	colleges.									
Mizoram	7 buildings.	2023	Yes	Yes List of post identified suitable for persons with disabilities and appointment of Liaison officer.	Yes	yes	Not specified	Not mentioned	Not mentioned	Lack of information on utilization of funds and structure of transport and websites of the States.
Nagaland	16 buildings mainly government offices, girls hostels and government colleges.	Swabhi man in 2021	No	NO	No	Yes	Not Specified	No	No	All other accessibility issues mentioned in the first access audit report of the buildings prepared in 2016 still exist , no action has been taken to resolve those issues and making the buildings more accessible for people.
Odisha	Not	August	No	No	No	No	Not	no	Yes	No initiative and

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
	Specified	2013					Specified			awareness programme was held post covid. Only one workshop held in 2023 to create awareness in government schools
Punjab	108 buildings mainly hospitals, colleges and government buildings.	Not Specified	No	Yes	No	No	Yes allocation of money has been classified under various heads in the report	No	No	There is a lack of clarity on the Number of buildings identified and retrofitted by the State.
Telangana	Not Specified	2021	No	No	Yes	Yes	Not Specified	Yes	No	No specific information provided on the progress made after the audit reports. Only 1 bus stand has been audited for accessibility in the transport sector.
West Bengal	21 buildings mainly government	Yes	NO	Yes	1486 buses have been made	yes	Not specified	25 personnel from 2017 were appointed through	21 state government websites were	Updates required on progress of reading buildings for retrofitting

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
	colleges and judicial academy and government offices.				accessible.			diploma sign language interpreter	found accessible	Remaining state departments need to provide information on accessible websites
UNION TERRITORIES										
Andaman and Nicobar Islands	64 buildings mainly government offices and government schools..	2019 Phase-1 and 2 done	Yes	No	Yes	Yes	Not Specified	Yes	Yes 15 websites are accessible.	No updates on the progress done after the audits. No information specified on the employment opportunities.
Chandigarh	97 buildings were identified for retrofitting. No further update done.	Access audits for 20 buildings were done.	No	No	No	No	Yes	Yes	No	No updates given on other aspects of accessibility. The progress report after the access audit is not attached.
Delhi	Not Specified	Yes	Yes	Yes	Yes	No	Not Specified	Yes	No	Several grievances and representations were addressed where people having UDID cards were

Name	No of buildings retrofitted	Access Audits Done (Year)	Equal Opportunity Policy Followed	Follow Ups Done After the Access Audit	Accessibility done by the Transport department	Accessibility work done by P And RD department in respect of Infrastructure Facility	Allocation of money from the center	Implementation of the Awareness Programme and Initiatives Taken	Digital Infrastructure	Recommendations
										not able to avail concession and facilities. The State further promises to adhere to the recommendation made in the report.

Table II.3

States & Union Territories	No. of buildings retrofitted	Majority of retrofitted buildings
Arunachal Pradesh	17	Mainly schools & secretariat
Assam	20	Not specified
Bihar	14	Not specified
Chhattisgarh	42	Not specified
Jharkhand	14	Mainly Govt. colleges, Govt. schools & secretariat
Haryana	Phase-I --> 54 Phase-II --> 02 Phase-III --> 26	Not specified
Kerala	Phase-I --> 28 Phase-II --> 06	Mainly schools, Govt. colleges, district panchayats & hospitals
Meghalaya	21	Mainly hospitals, Govt. schools, Govt. offices & Govt. colleges
Mizoram	7	Not specified
Nagaland	16	Mainly girls' hostels, Govt. colleges & Govt. offices
Odisha	Not specified	Not specified
Punjab	108	Mainly hospitals, colleges & Govt. buildings
Telangana	Not specified	Not specified
West Bengal	21	Mainly Govt. offices, Govt. colleges & Judicial academy
Andaman and Nicobar Islands	64	Mainly Govt. offices & Govt. schools
Chandigarh	97	Not specified
Delhi	Not specified	Not specified

Annexure II

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Reports From the States and Union Territories

To access these reports, [click here](#).

State Reports

1. Arunachal Pradesh
2. Assam
3. Bihar
4. Chandigarh
5. Chhattisgarh
6. Haryana
7. Jharkhand
8. Kerala
9. Meghalaya
10. Mizoram
11. Nagaland
12. Odisha
13. Punjab
14. West Bengal

Union Territories Reports

1. Andaman and Nicobar Islands
2. Chandigarh
3. Delhi

Reports Submitted by Experiential Experts, Subject Experts, DPOs and NGOs in the order in which they are referred to in the report

To access these reports, [click here](#).

1. **Ability Development and Inclusion (AADI); Muskaan, Action for Autism and National Platform for the Rights of the Disabled and Surbhi Meshram** facilitated the filing of affidavits by the following persons with cerebral palsy, intellectual disability, autism and sickle cell disease recording their life experiences.

Persons who have recorded their testimonies are:

1. Aditi Sowmyanarayan
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3. Rachelle
4. Shayonee Dasgupta
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44. Ankit Kumar
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46. Deva Barai
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48. Hardik
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12. Fourth Wave Foundation, Karnataka
13. Gandhi Development Foundation, Manipur
14. Handicapped Development Foundation, Manipur
15. Humanity Welfare Organisation Helpline, Jammu & Kashmir
16. India Education Collective, Uttar Pradesh
17. Indian Association of Muscular Dystrophy, Himachal Pradesh
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22. Madhya Pradesh Viklang Sahayta Samiti, Madhya Pradesh
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